NCE EXAM PREP

Practice Exam 11

Domain 5

Indirect Client Care

21 Practice Questions | 28 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 02. Use the suggested 28-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> Bao receives prescription instructions written in English but prefers health information in Vietnamese. He tells his counselor that he is unsure whether to take the medication with food. He has no acute symptoms, and an authorization permits communication with the pharmacy and prescriber.

What should the counselor do?

A) Document Bao's uncertainty and coordinate qualified Vietnamese language support with the pharmacist or prescriber for the medication instructions

B) Enter the instructions into a general translation application and explain the resulting directions to Bao

C) Infer the dosing instruction from the picture on the label and tell Bao what it most likely means

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Question 2

Which content in a transfer summary most directly supports continuity with the next counselor?

A) A chronological copy of every progress note, including material unrelated to current care

B) Current goals, interventions and responses, important client preferences, and pending follow-up needs

C) The diagnosis and appointment count without information about the client's current plan

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Question 3

> A counselor will meet Renee after she completes a six-week residential substance-use program. The discharge summary describes abstinence as the treatment goal, while the counselor's pre-admission notes document Renee's preference for a harm-reduction approach. No current outpatient goal has been confirmed.

What is the best preparation for the first post-discharge session?

A) Adopt the discharge goal because the residential program had the most recent contact with Renee

B) Review both records, note the difference, and prepare to ask Renee about her current goals and transition needs

C) Disregard the discharge summary because the outpatient record established Renee's original preference

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Question 4

> A primary care social worker referred Tasha for "anger management." During intake, Tasha describes caregiver strain and says her goals are to obtain respite and communicate more effectively with her father. Tasha has authorized the counselor to send the social worker a brief summary of engagement and goals, but not session notes.

What should the counselor communicate to the social worker?

A) Tasha's identified concerns and goals, along with any relevant coordination need permitted by the authorization

B) Confirmation that Tasha has an anger problem because that was the stated reason for referral

C) The complete intake narrative so the social worker can determine which account is more accurate

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Question 5

> Darius, a single father seeking help for gambling, wants a secular support group that he can reach by bus and attend while his child is in supervised care. The counselor has several recently verified programs in the referral directory.

How should the counselor narrow the referral options?

A) Recommend the closest program because distance is the most objective measure of access

B) Compare the verified programs with Darius's stated criteria and let him choose among the feasible options

C) Select a program based on which participant demographics seem most similar to Darius's background

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Question 6

> Nolan has low vision and tells his counselor that he sometimes skips an evening medication because two pharmacy bottles have nearly identical small-print labels. He reports no adverse symptoms. A written authorization permits coordination with the pharmacy and prescriber.

Which action is most appropriate for the counselor?

A) Encourage Nolan to rely on the slight cap-color difference and revisit the issue if an error occurs

B) Suggest taking both medications in the morning so Nolan does not need to identify them later

C) Document the access barrier and help Nolan request verified accessible labeling or packaging through the pharmacy and prescriber

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Question 7

> Avery, a high school student in a military family, will move to another state in three weeks. Avery and a parent have authorized coordination with the receiving school counselor. Avery wants continued support for lunchtime anxiety and values being able to visit the counseling office briefly when overwhelmed.

Which coordination action best supports the school transition?

A) Forward the entire school counseling file without discussing which supports Avery wants continued

B) Wait until after the move so the receiving counselor can assess Avery without prior expectations

C) Prepare a concise handoff of effective supports, Avery's preferences, and pending needs, and arrange an introductory contact

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Question 8

When a client and counselor revise a treatment goal, which documentation best supports continuity of care?

A) Replace the original goal with the new goal so later readers see only the current plan

B) Record the change, the client's reason for it, the revised goal, and the related next step

C) Retain the original goal without revision until the client's diagnosis also changes

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Question 9

> Mateo accepted a referral to a caregiver-support group that the counseling center's directory listed as Spanish-language. When he called, the program said the group is currently offered only in English, so he did not enroll.

How should the counselor respond to the failed linkage?

A) Verify the program change, update the directory, and help Mateo locate a currently verified Spanish-language option that fits his preferences

B) Encourage Mateo to attend the English-language group because the topic is more important than the language used

C) Keep the directory entry unchanged until the program revises its public website

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Question 10

> In a relapse-prevention group, Maya described a recent craving, practiced urge surfing, and planned to call her recovery sponsor if the craving returned. Other members also shared personal experiences during the session.

What should the counselor include in Maya's individual progress note?

A) A detailed summary of every member's disclosure to show how the group influenced Maya

B) Only the statement "attended and participated" because group content cannot be documented individually

C) Maya's relevant participation, response to the intervention, and follow-up plan without identifying other members' disclosures

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Question 11

> Nadine asks her counselor to coordinate with an elder from her cultural community as an additional source of support. She authorizes limited care-planning communication and says she wants to continue counseling while also participating in a ceremony the elder recommends.

What is the counselor's best coordination approach?

A) Decline collaboration because a nonclinical cultural support cannot contribute to a counseling plan

B) Ask the elder to direct the treatment plan so the services remain culturally consistent

C) Clarify with Nadine the roles and information she wants shared, then coordinate complementary supports within the authorization

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Question 12

> Jordan missed two counseling visits. The prior note states that rotating work shifts were creating scheduling problems and that Jordan chose text reminders. The scheduling log shows that reminders were sent, but it does not indicate why Jordan missed the visits. Another appointment is now scheduled.

How should the counselor prepare for the next meeting?

A) Review the attendance and contact information and prepare neutral questions about barriers and scheduling preferences

B) Add "resistant to treatment" to the case summary because the reminders did not improve attendance

C) Plan to restate the treatment goals without discussing the missed visits or the rotating shifts

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Question 13

During an interpreted counseling session, a qualified interpreter says that a counseling term has no direct equivalent in the client's preferred language. What should the counselor do?

A) Ask the interpreter to select the closest single word without further discussion

B) Omit the concept so the difference in language does not interrupt the session

C) Explain the concept in plain, concrete language, allow the interpreter to convey the meaning, and check the client's understanding

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Question 14

> Ms. Patel lives alone and has authorized her counselor to coordinate with a meal-delivery program. The program reports that several meals were returned unopened. Ms. Patel says new dental pain makes solid food difficult to eat and emphasizes that she wants to remain in her home.

Which coordination plan best reflects the available information?

A) Tell the program that Ms. Patel is refusing meals and needs closer compliance monitoring

B) With Ms. Patel's input, relay the functional barrier and coordinate temporary texture-appropriate meals and access to dental support

C) Recommend an assisted-living placement because returning meals indicates that independent living is no longer feasible

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Question 15

> Luis, an adult with a mild intellectual disability, is considering supported-housing options. He has authorized coordination with his sister, an independent-living advocate, and a case manager. During planning calls, his sister answers questions quickly, and Luis stops participating. Luis tells the counselor that he wants more time to answer for himself.

How should the counselor prepare for the next coordination meeting?

A) Ask the sister to present Luis's preferences because she can communicate them more efficiently

B) Exclude the other participants so Luis will not experience pressure during planning

C) Confirm Luis's preferred meeting format and structure the meeting to give him response time before others contribute

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Question 16

> Before signing a templated progress note, a counselor notices that an automatically carried-forward field says "employment unchanged." During the session, the client reported starting a job and changed future appointment times to fit the new schedule.

What should the counselor do?

A) Correct the field with the individualized information and verify the other carried-forward content before signing

B) Sign the note as generated and add a correction only if the employment change later affects treatment

C) Keep the standardized field because consistency across notes is more important than a nonclinical detail

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Question 17

> Mei, an autistic adult living in a group home, has authorized coordination with residential staff. Staff describe "meltdowns" during dinner. Mei says the dining room noise is overwhelming and asks to eat in a quieter area. No one has yet tracked what happens before or after the episodes.

What should the counselor facilitate?

A) A joint review of specific observations and Mei's sensory experience, followed by an agreed support trial and tracking plan

B) Acceptance of the staff's description as the clinical explanation because they observe the episodes directly

C) Separate plans for Mei and staff so neither party's interpretation influences the other's response

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Question 18

> A building evacuation interrupts a counseling session before the counselor completes the progress note. The counselor records brief contemporaneous facts and safely finishes the full note the next morning.

How should the completed documentation represent the timing?

A) Use the original session time as the note-completion time because the content concerns that session

B) Reconstruct exact quotations from memory so the delayed note appears as detailed as a routine entry

C) Identify the entry as completed later, use the actual completion time, and distinguish known facts from any limited recollection

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Question 19

A referral program temporarily pauses new intakes. Which directory update is most useful?

A) Delete the program permanently so no counselor can refer to a service that is currently unavailable

B) Record the verification date, current pause, expected recheck point, and any available alternatives

C) Leave the program marked active because temporary capacity changes are too frequent to track

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Question 20

> Kiara, a new parent, receives counseling and support from a home-visiting nurse, a lactation consultant, and a food-assistance program. She has authorized the providers to coordinate. Kiara reports repeating the same history to each provider and missing visits because appointments overlap.

What would be the most useful coordination response?

A) Have each provider keep an independent plan and ask Kiara to resolve scheduling conflicts as they occur

B) Establish client-approved shared priorities, designate a coordination contact, and align the appointment schedule

C) Ask the home-visiting nurse to control all service planning because that provider sees Kiara at home

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Question 21

> Amara prefers to receive counseling in Tigrinya, and the clinic has scheduled a qualified interpreter for her intake. Before the appointment, the interpreter asks the counselor what information would be useful for preparation. The counselor has not yet met Amara.

What is the counselor's best response?

A) Briefly explain the session purpose and likely terminology, confirm the interpreter's role, and plan to speak directly with Amara

B) Provide Amara's full referral record and ask the interpreter to summarize the relevant history before the session

C) Ask the interpreter to conduct the initial symptom screening alone and report the results to the counselor

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: A

Correct Answer: Document Bao's uncertainty and coordinate qualified Vietnamese language support with the pharmacist or prescriber for the medication instructions

Explanation: Medication directions should come from the dispensing or prescribing professional in a language Bao can understand. The counselor can document the access problem and coordinate qualified language support without interpreting the dose. A general translation application may produce unsafe ambiguity in medication instructions.

Question 2 — Correct: B

Correct Answer: Current goals, interventions and responses, important client preferences, and pending follow-up needs

Explanation: These elements tell the next counselor what is active, what has been tried, how the client responded, and what still requires attention. A complete note dump can bury the current plan in unrelated history. Diagnosis and visit count alone do not support an informed continuation of care.

Question 3 — Correct: B

Correct Answer: Review both records, note the difference, and prepare to ask Renee about her current goals and transition needs

Explanation: Both records contain relevant but time-bound information, and neither establishes Renee's present outpatient goal. Recognizing the difference prepares the counselor to clarify rather than assume. Automatically adopting or rejecting the discharge goal would place a record above Renee's current perspective.

Question 4 — Correct: A

Correct Answer: Tasha's identified concerns and goals, along with any relevant coordination need permitted by the authorization

Explanation: The authorized feedback should accurately represent Tasha's assessed concerns and client-defined goals. A referral label is not a confirmed clinical fact, particularly when the intake provides a different context. Sending the full intake would exceed the limited summary Tasha authorized.

Question 5 — Correct: B

Correct Answer: Compare the verified programs with Darius's stated criteria and let him choose among the feasible options

Explanation: Referral fit depends on the combination of service approach, transportation, scheduling, and childcare that Darius identified. Comparing verified options preserves choice while screening out services he cannot realistically use. Geographic closeness alone does not address all of the access requirements.

Question 6 — Correct: C

Correct Answer: Document the access barrier and help Nolan request verified accessible labeling or packaging through the pharmacy and prescriber

Explanation: The problem is reliable access to medication information, not a need for the counselor to change the regimen. Accessible labels or packaging can be verified by the professionals responsible for dispensing and prescribing the medication. Relying on an uncertain visual cue could increase the chance of a medication error.

Question 7 — Correct: C

Correct Answer: Prepare a concise handoff of effective supports, Avery's preferences, and pending needs, and arrange an introductory contact

Explanation: A focused handoff gives the receiving counselor actionable continuity information and includes Avery's preferences. An introductory contact can reduce the gap created by the move. Forwarding the entire file without selecting relevant content may obscure the supports that matter most.

Question 8 — Correct: B

Correct Answer: Record the change, the client's reason for it, the revised goal, and the related next step

Explanation: This documentation preserves the treatment history while making the current direction and client involvement clear. Replacing the old goal without context obscures how the plan evolved. A goal can change without a diagnostic change.

Question 9 — Correct: A

Correct Answer: Verify the program change, update the directory, and help Mateo locate a currently verified Spanish-language option that fits his preferences

Explanation: Mateo's experience provides a concrete reason to recheck the directory and prevent the same failed referral for others. The counselor should also help restore the linkage using current, language-appropriate information. Asking Mateo to accept a service he cannot use effectively does not resolve the access barrier.

Question 10 — Correct: C

Correct Answer: Maya's relevant participation, response to the intervention, and follow-up plan without identifying other members' disclosures

Explanation: Maya's individual note should show what she did, how she responded, and what she plans next. Other members' detailed disclosures are not needed to document Maya's care. A generic attendance statement would omit clinically useful information about the intervention and response.

Question 11 — Correct: C

Correct Answer: Clarify with Nadine the roles and information she wants shared, then coordinate complementary supports within the authorization

Explanation: Nadine has asked for both forms of support and has authorized limited planning communication. Clarifying roles allows the counselor and elder to complement one another without surrendering the counseling plan or dismissing the cultural resource. The client's preferences define the purpose and boundaries of the coordination.

Question 12 — Correct: A

Correct Answer: Review the attendance and contact information and prepare neutral questions about barriers and scheduling preferences

Explanation: The record identifies a possible work-schedule barrier but does not explain the missed visits. Neutral preparation supports collaborative problem-solving based on updated information. Labeling Jordan resistant would convert an unexplained attendance pattern into an unsupported judgment.

Question 13 — Correct: C

Correct Answer: Explain the concept in plain, concrete language, allow the interpreter to convey the meaning, and check the client's understanding

Explanation: When a direct equivalent is unavailable, conveying the concept's meaning is more accurate than forcing a single-word substitution. A comprehension check helps the counselor determine whether the explanation worked. Omitting the concept would create an avoidable gap in the session.

Question 14 — Correct: B

Correct Answer: With Ms. Patel's input, relay the functional barrier and coordinate temporary texture-appropriate meals and access to dental support

Explanation: The returned meals have a specific, potentially remediable explanation that is consistent with Ms. Patel's goal of remaining at home. Coordinating food texture and dental access addresses the barrier without mislabeling her behavior. The available facts do not establish a need for residential placement.

Question 15 — Correct: C

Correct Answer: Confirm Luis's preferred meeting format and structure the meeting to give him response time before others contribute

Explanation: Coordination should make it possible for Luis to communicate his own preferences while still using the supports he authorized. Adjusting the meeting structure directly addresses the participation barrier he identified. Substituting his sister's voice would undermine supported decision-making rather than improve it.

Question 16 — Correct: A

Correct Answer: Correct the field with the individualized information and verify the other carried-forward content before signing

Explanation: Automatically carried-forward text must be checked against what actually occurred in the session. Correcting the employment field and reviewing the remaining template content prevents inaccurate information from propagating. Standardization does not justify retaining a statement known to be false.

Question 17 — Correct: A

Correct Answer: A joint review of specific observations and Mei's sensory experience, followed by an agreed support trial and tracking plan

Explanation: Specific observations and Mei's account can generate a testable support plan without treating a vague label as an explanation. Tracking the agreed trial will help the team evaluate whether the quieter setting changes the pattern. Separate plans would prevent the coordinated response the situation requires.

Question 18 — Correct: C

Correct Answer: Identify the entry as completed later, use the actual completion time, and distinguish known facts from any limited recollection

Explanation: Transparent timing and careful attribution preserve the note's accuracy after an unavoidable delay. The counselor can use contemporaneous facts without pretending the entry was completed during the session. Reconstructed exact quotations would imply a level of certainty the counselor may not have.

Question 19 — Correct: B

Correct Answer: Record the verification date, current pause, expected recheck point, and any available alternatives

Explanation: A time-stamped status lets counselors avoid a currently closed pathway while preserving information that may become useful again. A recheck point and alternatives make the entry actionable. Permanent deletion would lose a potentially valuable resource because of a temporary condition.

Question 20 — Correct: B

Correct Answer: Establish client-approved shared priorities, designate a coordination contact, and align the appointment schedule

Explanation: A shared plan and identified coordination contact reduce repeated storytelling and prevent avoidable scheduling conflicts. Kiara's approval keeps the coordination centered on her needs. Giving one provider unilateral control would not reflect the collaborative roles of the team.

Question 21 — Correct: A

Correct Answer: Briefly explain the session purpose and likely terminology, confirm the interpreter's role, and plan to speak directly with Amara

Explanation: A focused pre-session briefing helps the qualified interpreter anticipate terminology while preserving the counselor's direct relationship with Amara. Because the counselor has not assessed Amara, a full history or interpreter-led screening would be both unnecessary and role-confusing. The counselor remains responsible for conducting the intake.
